



Tests you can trust

Name : Vineet Beniwal(25Y/M)

Date : 24 May, 2026

Test Asked : Aarogyam Basic 2 Special

Report Status : Complete Report



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NABL From 2005[#]



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CAP From 2011⁻



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Accurate & Reliable^{*}



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First National Diagnostic Chain to have 100% of its Labs with NABL Accreditation[#]

Patient Name

: VINEET BENIWAL(25Y/M)

Referred By

: SELF

Address

: ADDRESS WITH NEAREST LANDMARK : E - 34 SAI DHAM RAJ CITY LONI
GHAZIABAD PINCODE : 201102

Tests Done

: AAROGYAM BASIC 2 SPECIAL

Report Availability Summary

Note: Please refer to the table below for status of your tests.

 **8** Ready

 **0** Ready with Cancellation

 **0** Processing

 **0** Cancelled in Lab

TEST DETAILS	REPORT STATUS
AAROGYAM BASIC 2 SPECIAL	Ready 
LIPID PROFILE	Ready 
HEMOGRAM - 6 PART (DIFF)	Ready 
T3-T4-USTSH	Ready 
HbA1c	Ready 
KIDPRO	Ready 
COMPLETE URINE ANALYSIS	Ready 
IRON DEFICIENCY PROFILE	Ready 
LIVER FUNCTION TESTS	Ready 

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Tests Done : AAROGYAM BASIC 2 SPECIAL

Tests Outside Reference Range

Note: Please refer to the table below for tests outside reference range.

Test Name	Observed Value	Units	Bio. Ref. Interval.
LIPID			
HDL / LDL RATIO	0.32	Ratio	> 0.40
LDL CHOLESTEROL - DIRECT	131	mg/dL	< 100
TRIG / HDL RATIO	4.28	Ratio	< 3.12
TRIGLYCERIDES	182	mg/dL	< 150
LIVER			
BILIRUBIN (INDIRECT)	0.93	mg/dL	0-0.9
THYROID			
TSH - ULTRASENSITIVE	5.58	μIU/mL	0.54-5.30

Patient Name : VINEET BENIWAL(25Y/M)
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Address : ADDRESS WITH NEAREST LANDMARK : E - 34 SAI DHAM RAJ
CITY LONI GHAZIABAD PINCODE : 201102

Sample Collected on (SCT) : 24 May 2026 08:57
Sample Received on (SRT) : 24 May 2026 17:12
Report Released on (RRT) : 24 May 2026 18:23
Sample Type | Barcode : EDTA Whole Blood | FD315330

TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c	H.P.L.C	5.2	%

Bio. Ref. Interval. :

As per ADA Guidelines

Below 5.7% : Normal
5.7% - 6.4% : Prediabetic
≥6.5% : Diabetic

Guidance For Known Diabetics

Below 6.5% : Good Control
6.5% - 7% : Fair Control
7.0% - 8% : Unsatisfactory Control
≥8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG) CALCULATED 103 mg/dL

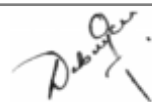
Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HbA1c values

Please correlate with clinical conditions.

Tests Done : AAROYAM BASIC 2 SPECIAL



Dr Debanjan
Bhattacharya DNB(Path)



Dr Preety Garg
MD(Path)



Scan QR to verify(valid for
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TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	SLS-Hemoglobin Method	15.8	g/dL	13.0-17.0
Hematocrit (PCV)	CPH Detection	49	%	40.0-50.0
Total RBC	HF & EI	5.44	X 10 ⁶ /μL	4.5-5.5
Mean Corpuscular Volume (MCV)	Calculated	90.1	fL	83.0-101.0
Mean Corpuscular Hemoglobin (MCH)	Calculated	29	pq	27.0-32.0
Mean Corp.Hemo. Conc (MCHC)	Calculated	32.2	g/dL	31.5-34.5
Red Cell Distribution Width - SD (RDW-SD)	Calculated	45.1	fL	39-46
Red Cell Distribution Width (RDW - CV)	Calculated	13.8	%	11.6-14
RED CELL DISTRIBUTION WIDTH INDEX (RDWI)	Calculated	228.6	-	*Refer Note below
MENTZER INDEX	Calculated	16.6	-	*Refer Note below
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	9.5	X 10 ³ / μL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	62.8	%	40-80
Lymphocytes Percentage	Flow Cytometry	30.5	%	20-40
Monocytes Percentage	Flow Cytometry	3.7	%	2-10
Eosinophils Percentage	Flow Cytometry	2.2	%	1-6
Basophils Percentage	Flow Cytometry	0.5	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.3	%	0-0.5
Nucleated Red Blood Cells %	Flow Cytometry	0.01	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	5.97	X 10 ³ / μL	2.0-7.0
Lymphocytes - Absolute Count	Calculated	2.9	X 10 ³ / μL	1.0-3.0
Monocytes - Absolute Count	Calculated	0.35	X 10 ³ / μL	0.2 - 1.0
Basophils - Absolute Count	Calculated	0.05	X 10 ³ / μL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.21	X 10 ³ / μL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.03	X 10 ³ / μL	0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10 ³ / μL	0.0-0.5
PLATELET COUNT	HF & EI	255	X 10 ³ / μL	150-410
Mean Platelet Volume (MPV)	Calculated	11.5	fL	6.5-12
Platelet Distribution Width (PDW)	Calculated	14.5	fL	9.6-15.2
Platelet to Large Cell Ratio (PLCR)	Calculated	36.2	%	19.7-42.4
Plateletcrit (PCT)	Calculated	0.29	%	0.19-0.39

Remarks : Alert!!! Predominantly normocytic normochromic with ovalocytes. Platelets:Appear adequate in smear.

*Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.

Method : Fully automated bidirectional analyser

(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height)

Tests Done : AAROGYAM BASIC 2 SPECIAL

Dr Debanjan
Bhattacharya DNB(Path)

Dr Preety Garg
MD(Path)

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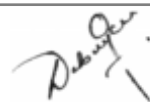
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Referred By : SELF
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CITY LONI GHAZIABAD PINCODE : 201102

Sample Collected on (SCT) : 24 May 2026 08:57
Sample Received on (SRT) : 24 May 2026 17:12
Report Released on (RRT) : 24 May 2026 20:54
Sample Type | Barcode : SERUM | FA474430

TEST NAME	TECHNOLOGY	VALUE	UNITS
IRON Bio. Ref. Interval. : Male : 65 - 175 Female : 50 - 170 Method : Ferrozine method without deproteinization	PHOTOMETRY	106	µg/dL
TOTAL IRON BINDING CAPACITY (TIBC) Bio. Ref. Interval. : Male: 225 - 535 µg/dl Female: 215 - 535 µg/dl Method : Spectrophotometric Assay	PHOTOMETRY	352	µg/dL
% TRANSFERRIN SATURATION Bio. Ref. Interval. : 13 - 45 Method : Derived from IRON and TIBC values	CALCULATED	30	%
UNSAT.IRON-BINDING CAPACITY(UIBC) Bio. Ref. Interval. : 162 - 368 Method : SPECTROPHOTOMETRIC ASSAY	PHOTOMETRY	246.9	µg/dL

Please correlate with clinical conditions.

Tests Done : AAROGRAM BASIC 2 SPECIAL



Dr Debanjan
Bhattacharya DNB(Path)



Dr Preety Garg
MD(Path)

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
TOTAL CHOLESTEROL	PHOTOMETRY	193	mg/dL	< 200
HDL CHOLESTEROL - DIRECT	PHOTOMETRY	43	mg/dL	40-60
LDL CHOLESTEROL - DIRECT	PHOTOMETRY	131	mg/dL	< 100
TRIGLYCERIDES	PHOTOMETRY	182	mg/dL	< 150
TC/ HDL CHOLESTEROL RATIO	CALCULATED	4.5	Ratio	3 - 5
TRIG / HDL RATIO	CALCULATED	4.28	Ratio	< 3.12
LDL / HDL RATIO	CALCULATED	3.1	Ratio	1.5-3.5
HDL / LDL RATIO	CALCULATED	0.32	Ratio	> 0.40
NON-HDL CHOLESTEROL	CALCULATED	150.94	mg/dL	< 160
VLDL CHOLESTEROL	CALCULATED	36.42	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

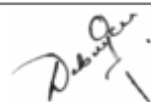
CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
LDL - Direct Measure
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDH - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

Tests Done : AAROGYAM BASIC 2 SPECIAL



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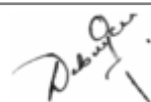
TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
ALKALINE PHOSPHATASE	PHOTOMETRY	101.2	U/L	45-129
BILIRUBIN - TOTAL	PHOTOMETRY	1.12	mg/dL	0.3-1.2
BILIRUBIN -DIRECT	PHOTOMETRY	0.19	mg/dL	0 - 0.20
BILIRUBIN (INDIRECT)	CALCULATED	0.93	mg/dL	0-0.9
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	48.6	U/L	< 55
ASPARTATE AMINOTRANSFERASE (SGOT)	PHOTOMETRY	22.11	U/L	< 35
ALANINE TRANSAMINASE (SGPT)	PHOTOMETRY	25.97	U/L	< 45
SGOT / SGPT RATIO	CALCULATED	0.85	Ratio	< 2
PROTEIN - TOTAL	PHOTOMETRY	7.61	gm/dL	5.7-8.2
ALBUMIN - SERUM	PHOTOMETRY	4.7	gm/dL	3.2-4.8
SERUM GLOBULIN	CALCULATED	2.91	gm/dL	2.5-3.4
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.62	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method
BILT - Diazonium salt DPD method
BILD - Diazonium salt DPD method
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Modified IFCC method
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
OT/PT - Derived from SGOT and SGPT values.
PROT - Biuret Method
SALB - Albumin Bcg¹method (Colorimetric Assay Endpoint)
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

Tests Done : AAROGRAM BASIC 2 SPECIAL



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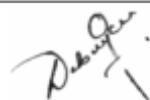
TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
BLOOD UREA NITROGEN (BUN)	PHOTOMETRY	8.4	mg/dL	7.94 - 20.07
CREATININE - SERUM	PHOTOMETRY	0.89	mg/dL	0.72-1.18
BUN / SR.CREATININE RATIO	CALCULATED	9.44	Ratio	9:1-23:1
UREA (CALCULATED)	CALCULATED	17.98	mg/dL	Adult : 17-43
UREA / SR.CREATININE RATIO	CALCULATED	20.2	Ratio	< 52
CALCIUM	PHOTOMETRY	9.7	mg/dL	8.8-10.6
URIC ACID	PHOTOMETRY	4.9	mg/dL	4.2 - 7.3

Please correlate with clinical conditions.

Method :

BUN - Kinetic UV Assay.
SCRE - Creatinine Enzymatic Method
B/CR - Derived from serum Bun and Creatinine values
UREAC - Derived from BUN Value.
UR/CR - Derived from UREA and Sr.Creatinine values.
CALC - Arsenazo III Method, End Point.
URIC - Uricase / Peroxidase Method

Tests Done : AAROGYAM BASIC 2 SPECIAL



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Bhattacharya DNB(Path)



Dr Preety Garg
MD(Path)

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	E.C.L.I.A	120	ng/dL	80-200
TOTAL THYROXINE (T4)	E.C.L.I.A	7.48	µg/dL	4.8-12.7
TSH - ULTRASENSITIVE	E.C.L.I.A	5.58	µIU/mL	0.54-5.30

Comments : IF NOT ON DRUGS SUGGESTED FT3 & FT4 ESTIMATION

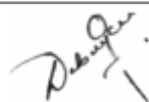
The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3,T4 - Fully Automated Electrochemiluminescence Compititive Immunoassay
USTSH - Fully Automated Electrochemiluminescence Sandwich Immunoassay

Disclaimer :Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

Tests Done : AAROGYAM BASIC 2 SPECIAL



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Sample Type | Barcode : SERUM | FA474430

TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR) Bio. Ref. Interval. :-	CALCULATED	122	mL/min/1.73 m2

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease
<15 : Kidney Failure

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

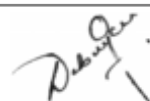
Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

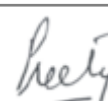
Please correlate with clinical conditions.

Method:- 2021 CKD EPI Creatinine Equation

Tests Done : AAROGYAM BASIC 2 SPECIAL



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Sample Type | Barcode : URINE | EX531284

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
Complete Urinogram				
<u>Physical Examination</u>				
VOLUME	Visual Determination	3	mL	-
COLOUR	Visual Determination	PALE YELLOW	-	Pale Yellow
APPEARANCE	Visual Determination	CLEAR	-	Clear
SPECIFIC GRAVITY	pKa change	1.020	-	1.003-1.030
PH	pH indicator	6	-	5-8
<u>Chemical Examination</u>				
URINARY PROTEIN	PEI	ABSENT	mg/dL	Absent
URINARY GLUCOSE	GOD-POD	ABSENT	mg/dL	Absent
URINE KETONE	Nitroprusside	ABSENT	mg/dL	Absent
URINARY BILIRUBIN	Diazo coupling	ABSENT	mg/dL	Absent
UROBILINOGEN	Diazo coupling	Normal	mg/dL	<=0.2
BILE SALT	Hays sulphur	ABSENT	-	Absent
BILE PIGMENT	Ehrlich reaction	ABSENT	-	Absent
URINE BLOOD	Peroxidase reaction	ABSENT	-	Absent
NITRITE	Diazo coupling	ABSENT	-	Absent
LEUCOCYTE ESTERASE	Esterase reaction	ABSENT	-	Absent
<u>Microscopic Examination</u>				
MUCUS	Microscopy	ABSENT	-	Absent
RED BLOOD CELLS	Microscopy	1	cells/HPF	0-2
URINARY LEUCOCYTES (PUS CELLS)	Microscopy	1	cells/HPF	0-5
EPITHELIAL CELLS	Microscopy	ABSENT	cells/HPF	0-5
CASTS	Microscopy	ABSENT	-	Absent
CRYSTALS	Microscopy	ABSENT	-	Absent
BACTERIA	Microscopy	ABSENT	-	Absent
YEAST	Microscopy	ABSENT	-	Absent
PARASITE	Microscopy	ABSENT	-	Absent

(Reference : *PEI - Protein error of indicator, *GOD-POD - Glucose oxidase-peroxidase)

~~ End of report ~~

Tests Done : AAROGYAM BASIC 2 SPECIAL



Preety



Dr Debanjan
Bhattacharya DNB(Path)

Dr Preety Garg
MD(Path)

Scan QR to verify(valid for
30 days from release time)

CONDITIONS OF REPORTING

- ✓ The reported results are for information and interpretation of the referring doctor only.
- ✓ It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- ✓ Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- ✓ Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- ✓ Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- ✓ This report is not valid for medico-legal purpose.
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- ✓ Thyrocare Discovery video link :- <https://youtu.be/nbdYeRgYyQc>

EXPLANATIONS

- ✓ Majority of the specimen processed in the laboratory are collected by Pathologists and Hospitals we call them as "Clients".
- ✓ **Name** - The name is as declared by the client and recored by the personnel who collected the specimen.
- ✓ **Ref.Dr** - The name of the doctor who has recommended testing as declared by the client.
- ✓ **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- ✓ **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- ✓ **SCP** - Specimen Collection Point - This is the location where the blood or specimen was collected as declared by the client.
- ✓ **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- ✓ **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- ✓ **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- ✓ **Reference Range** - Means the range of values in which 95% of the normal population would fall.

SUGGESTIONS

- ✓ Values out of reference range requires reconfirmation before starting any medical treatment.
- ✓ Retesting is needed if you suspect any quality shortcomings.
- ✓ Testing or retesting should be done in accredited laboratories.
- ✓ For suggestions, complaints, clinical support or feedback, write to us at customersupport@thyrocare.com or call us on **022-3090 0000**

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* As per survey on doctors' perception of laboratory diagnostics (IJARIIT, 2023), -Mumbai Reference Lab is CAP Accredited